Anxiety and Related Disorders

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Introduction to Psychological Disorders

Module 3
Opening to Anxiety Disorders

Module 4
Components of Anxiety

Module 5
Disorders that Involve Anxiety Part 1

Generalized Anxiety Disorders
- Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Generalized Anxiety Disorder
  Reading • 10 min
- Check Your Understanding: Generalized Anxiety Disorder
  Practice Assignment • Grade: 100%

Panic Disorder
- Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Panic Disorder
  Reading • 10 min
- Check Your Understanding: Panic Disorder
  Practice Assignment • Grade: 80%

Specific Phobias
- **Diagnostic Criteria and Prevalence for Specific Phobias**
  Reading • 15 min
- Contributing Factors for Specific Phobias
  Reading • 10 min
- Classical Conditioning of Specific Phobias
  Video • 2 min
- Operant Conditioning and Negative Reinforcement of Specific Phobias
  Video • 3 min
- Cultural Issues for Specific Phobias
  Reading • 2 min
- Check Your Understanding: Specific Phobias
  Practice Assignment • 6 min

Key Takeaways
- Key Takeaways: Disorders that Involve Anxiety Part 1
  Reading • 10 min

Module 5 Quiz
- Module Quiz: Disorders that Involve Anxiety Part 1
  Graded Assignment • 10 min

Module 6
Disorders that Involve Anxiety Part 2

Module 7
Conclusion

Module 8
Cumulative Quiz

Module 9
Course Resources

Module 10
PsycLearn Essentials APA Student Resources

Diagnostic Criteria and Prevalence for Specific Phobias

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Specific Phobia Example

Scenario

"Lights out, campers!"

"Hey! Cabin 2, stop playing with your flashlights and go to bed!"

It wasn't long before the muffled giggles from inside the cabins started to subside. The campers had had a long and eventful day.

"I hope nobody sneaks out tonight," the camp counselor thought to himself as he settled into his hammock outside the cabins.

Suddenly, there was a scream from Cabin 2. A young camper, Miguel, rushed out screaming, tears streaming down his face.

"There is a spider, I saw a spider. I hate spiders!" Miguel cried out between gasps.

The counselor was going to send Miguel back to his cabin, but then took a quick look at the child's face and realized that Miguel wasn't just playing up his fear in an attempt to delay lights out. Miguel was terrified!

Seeking to calm the hyperventilating camper, the counselor told him that spiders are good to have around because they eat bugs, and that spiders are more afraid of him than he is of them. Despite the counselor's good intentions, every mention of spiders only made Miguel more scared. Ultimately, the counselor took Miguel to the nurse's office and, after convincing him that there were no spiders anywhere near the cabin, Miguel finally fell asleep.

The next morning, Miguel told the nurse about his fear of spiders:

"One time, I was playing with some empty flower pots in our shed. There must have been a spider's nest in there and all of a sudden, dozens of spiders were crawling all over me. They started biting me and…" His voice trailed off and he closed his eyes to try to calm himself down.

End of Scenario

Diagnostic Criteria

Feeling fear from time to time is normal. Fear can be adaptive and even motivating in the right circumstances. However, individuals who have a **specific phobia** of an object, situation, or environment experience a persistent and irrational fear that is distressing and profoundly interferes with their ability to function. Common phobic stimuli include snakes, heights, and blood. Although specific phobias do focus on a single feared stimulus, most individuals who have a specific phobia are likely to suffer from at least one or more additional phobias (APA, 2013; LeBeau et al., 2010).

As you saw in Miguel's case, individuals who suffer from specific phobias find ways to avoid the phobic stimuli whenever possible. If this is not possible, they will attempt to endure the phobic stimuli while exhibiting marked distress. What is particularly striking about specific phobias is that the individual is often aware that their behavior and response to the phobic stimuli are irrational (Eaton et al., 2018). For example, if Miguel sees a spider inside a locked glass box, he would still experience a high degree of fear and try to escape the situation, despite knowing that he is perfectly safe.

Miguel was able to identify the experience that created his specific phobia. This is typical of most people with specific phobias, but in some cases people cannot point to a specific event that led to the phobia (Mineka & Öhman, 2002).

People have phobias of a wide range of objects and situations. Some of the most common are included in the following table, "Specific Phobia Examples."

Specific Phobia Categories

Category	Common Fears
Blood, injection, or injury	Seeing blood, receiving an injection, or seeing a person suffer an injury
Animals	Spiders and other insects, snakes, dogs
Specific natural environments	Heights, open water, severe weather, such as thunder and lightning
Specific situations	Enclosed spaces, driving on the highway, being in the dark, flying in a plane

Note. This table shows examples of each of DSM-5's categories for specific phobias.

Notice that there are several common fears not listed in the above table: the fear of public speaking and the fear of embarrassing yourself in public. These are examples of social phobias and are covered in our next section, on social anxiety disorder.

Prevalence

At one time or another, you might have casually said that you have a phobia of heights, spiders, dogs, or some other stimulus. Although many of us use the term "phobia" to describe our fears in casual conversation, only about 7% of the global population is diagnosed with a phobia in their lifetime (Eaton et al., 2018). Whereas rates in the United States and European countries range from 6%–9%, rates in Asia, Africa, and Latin America are typically 2–4% (APA, 2013). In general, prevalence rates are higher in middle and higher income countries and lower in lower income countries (Wardenaar et al., 2017).

Both worldwide and in the United States, women are approximately twice as likely to be diagnosed with a specific phobia as men (APA, 2022; Wardenaar et al., 2017). In the United States, Asian and Latino/a Americans have lower rates of specific phobias than White, Black, and Native Americans (APA, 2022).

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